



Patient Name: Connie Riney

DOB: 06/29/1949

DOS: 05/04/2023 10:30 AM

Chief Complaint - Knee

Left Knee:

LEFT KNEE REPLACEMENT. Patient denies left knee pain

Right Knee:

Right Knee Pain Present

Right Knee Pain Described As: Aching Sore Stiffness

The frequency of right knee pain is reported as daily.

Intensity of pain on a 0 to 10 VAS scale (0 being no pain and 10 being worst pain you have ever had) is reported to be 5/10 on VAS.

Patient reports right knee pain occurring for 3; year(s).

HPI Knee

The patient presents today for evaluation and treatment of above complaints. Patient denies any radiating symptoms.

The patient has experienced this pain before. The patient reports crepitus on active motion in the right knee. The patient reports morning stiffness in the right knee lasting less than 20-30 minutes. Patient's range of motion is decreased secondary to pain. Patient reports there was no trauma to the area. Patient reports using Ibuprofen 600mg every 6 hours as needed for six months and states no overall significant improvement. Patient reports using Tylenol 500 mg bid as needed for six months and states no overall significant improvement. Patient reports not having tried muscle relaxants for management of symptoms. Patient reports not having tried narcotic pain medication for management of symptoms. Patient has not tried cortisone injections secondary to concerns over adverse reactions and Patient refused cortisone injection due to the long term risk of tissue/cartilage damage. The patient attempted the following to manage symptoms: HEP for six months and states no overall significant improvement. Patient presents today for a 6 month f/u visit after receiving Trivisc November 2022, right knee. Patient reports decreased pain levels after receiving injections. Patient states the injections lasted 6 months and there was 75% improvement in symptoms such as pain, soreness, stiffness, and swelling. Patient returned to doing activities of daily living, mobility increased, and OTC medication usage such as Ibuprofen/Tylenol decreased. Patient reports increasing pain over the last week and the patient would like to do viscosupplementation again.

Improvement of Pain

Patient reports improvement of knee pain with the following:

Avoid/ Limit Activities

Changing Positions

Joint Supplements

Medications

Sitting

Viscosupplementation

Right Knee

Worsening Pain

Patient reports worsening of knee pain with the following Activities of Daily Living:

Bending

Exercise

Housework
In/ Out of Vehicle
Kneeling
Personal Hygiene Routine
Physical Activity
Self-Care
Shopping
Sitting
Standing
Squatting/ Stooping
Stairs
Travel
Up from Bed/ Chair
Walking

HPI: Patient reports having tried the following treatment in the past:

Brace
Heat
Ice
OTC Creams

Viscosupplementation

Laterality: Right Knee

Allergies: None

Medications:

Patient Reported Medications:

Montelukast 10mg
loratadine 10mg
Venlafaxine 75mg
Lisinopril 20-12.5mg
Carvedilol 12.5mg
Furosemide 20mg
Budesonide 150 4.5mcg
Albuterol 30mcg
Etodolac 400mg
Felodipine 5mg
Solifenacin 5mg
Pravastatin 80mg
Omeprazole 40mg

Patient is currently not taking blood thinners.

Patient Medical History (v2)

Past Medical History:

Past Medical History Inclusive of: No Significant Medical History Reported Arthritis

Vital Signs

Weight 250 lbs Height 5' 4", BMI 42.91 kg/m sq, Category

During the knee testing instability has been noted. Functional knee testing as noted below:

KNEE RADIOGRAPHIC FINDINGS REPORT

INDICATION: Knee Pain

Arthritis Knee Pain Centers
1019 Majestic Dr
Ste 270 D
Lexington KY 40513-1496

Knee Measurements

Knee Inspection v5

Physical Exam:

Right Knee Findings:

Varus Angulation Deformity

Palpation:

Right Knee Crepitus is present with palpation in right knee.

Right Knee Pain with Palpation

Patient exhibits right knee pain with palpation

Pain with palpation exhibited at Medial Joint Line

Severity of Pain Moderate

Pre-Patella Bursa Swelling

Right Knee

Pre-patella bursa swelling absent on today's exam

Palpable Warmth

Right Knee

No evidence of palpable warmth, erythema, or skin rash on today's exam.

Knee Range of Motion:

Right Knee

Range of Motion Decreased

Range of motion is limited secondary to pain in right knee.

Gait and Ambulation:

Altered Gait

Patient ambulation status is described as ambulatory without assisted devices

Physical Exam

Knee Functionality

Right Knee: Positive Valgus Stress Test

Right Knee: Positive Grinding Test

Diagnoses

Stiffness of right knee, not elsewhere classified - M25.661

Onset is 2021-07-09.

--CMD Migrated

Pain in right knee - M25.561

Onset is 2021-07-09.

--CMD Migrated

Difficulty in walking, not elsewhere classified - R26.2, Secondary diagnosis

Onset is 2017-12-14.

--CMD Migrated

Unilateral primary osteoarthritis, right knee - M17.11, Primary diagnosis

Onset is 2017-12-14.

--CMD Migrated

Plan:

Patient has been recommended the following for the right knee:

Injectable:

The patient will be treated with a series 5 viscosupplementation procedures for placement of Genvisc 850 (J7320) in the knee. This will be performed at a frequency of once per week for 5 weeks. The patient will receive a dosage of 25 mg / 2.5 ml per knee for each procedure. Each dose of Genvisc 850 will utilize a pre-filled syringe containing 2.5 ml of Genvisc 850. Based on this dosage, 25.0 mg of Genvisc 850 will be used per procedure for each knee treated. FDA-approved hyaluronic acid shows a statistically significant reduction in knee pain and increasing levels of mobility in the majority of patients treated when compared to placebo. Promising evidence shows that Genvisc 850 and other forms of hyaluronic acid may slow deterioration of joints. To ensure that the medical device is placed properly into the joint space, fluoroscopic guidance will be necessary. This will allow for examination to assess the condition of the knee and help to rule out possible contraindications or other pathology causing or contributing to the patient's pain. In order to ensure the device is properly placed specifically into the knee capsule itself, subsequent double confirmation of the device will be necessary as per FDA guidelines. In order to avoid the delicate and possibly damaged and inflamed tissues surrounding the capsule, an initial diagnostic arthrogram will be performed. Fluoroscopic guidance for device placement, followed by a therapeutic arthrogram will be performed to diminish possibilities of the prosthetic medical device being placed anywhere other than into the knee capsule. This will help to diminish complications and lessen local flare reactions in the tissues surrounding the knee joint. Evidence shows that up to 40% of putative knee joint injections miss the joint space completely.

1. Jackson, Douglas W., et al. Accuracy of Needle Placement into the Intra-Articular Space of the Knee. The Journal of Bone and Joint Surgery (American) 84:1522-1527 (2002)

Plan:

Patient has been recommended the following for the right knee: POC discussed with patient. Recommendation for continued use of knee brace provided at previous appointment to help off load pressure on the knees. Patient voiced understanding.

Patient has been recommended the following for the right knee:

Patient has been discharged with a comprehensive HEP.

Improvements were sustained for at least six months following completion of treatment

Conservative therapy for six months has included one or more of the following: HEP, Tylenol, IBU, Brace

Following treatment pain was reduced

Functionality: Patient's functionality has improved

Patient's range of motion is increased since completion of treatment

Goals:

The goals of our treatment plan are as follows:

Decrease Pain

Increase Endurance/ Stamina of Affected Area

Increase Pain Free Range of Motion

Stabalize the Joint

Follow Up:

Patient will return in 4-6 weeks following completion of treatment for reevaluation.

Knee X-Ray Findings/ Impression

The study was performed on: date of knee screening with the patient in a standing position. Technical: The radiographs are of readable quality. Radiological findings were reviewed at the time imaging was obtained.

OTHER X-RAY NOTES

No xray completed during today's visit. Reviewed previous xray results. Please see previous note/screening note for xray results.

Electronically signed by Bird, Shawna LeAnne, FNP. on Thursday, May 4, 2023 at 10:55 AM